SACTCHECK TREATMENT ASSESSMENT

# Capecitabine

**NCCP 00216 | 8**
SACTCheck v0.83.4

| TUMOUR GROUP | INDICATION | COURSE |
|---|---|---|
| **Gastrointestinal** | **Breast and colorectal cancer indications; 1250 mg/m^2 twice daily on days 1-14 every 21 days.** | **Course metadata not confirmed** |

| TREATMENT CONTEXT | ASSESSMENT ID | ASSESSMENT TIME |
|---|---|---|
| **Not entered** | **JSON-DEMO-001** | **16/10/2026, 23:34** |

ENCODED CRITERIA RESULT

## 1 encoded criterion requires clinical review

Review the highlighted protocol criteria and the current NCCP source before the final treatment decision.

## ENTERED VALUES AND ENCODED CRITERIA

Standard assessment

| DOMAIN | ENTERED VALUE | ENCODED CRITERION | RESULT |
|---|---|---|---|
| **Creatinine clearance category** | 30-50 mL/min | >= 30 | **Dose-modification pathway** |
| **Creatinine clearance category** | 30-50 mL/min | <= 50 | **Dose-modification pathway** |
| **Haemodialysis** | No | = Yes | **Criterion met** |

## COMPONENT-SPECIFIC PROTOCOL ACTIONS

Protocol-level guidance; verify prescribed dose/current level

**Capecitabine: Use 75% of documented starting dose - 75% of documented starting dose per administration; confirm prior reductions and pharmacy tablet banding**
Use 75% of the original capecitabine dose for CrCl 30-50 mL/min; apply tablet dose banding and consultant review. page 3 | Table 2

### UNASSESSED DOMAINS

ECOG performance status, Hypersensitivity to capecitabine or its excipients, Previous severe, unexpected fluoropyrimidine reaction, Brivudine now or within the previous four weeks, Pregnant, Breastfeeding, DPD status, Assessment timing (optional) + 19 more

Blank domains were not assumed normal.

### CLINICIAN DECISION

**Not documented**
Not documented

**CLINICAL DECISION SUPPORT - NOT TREATMENT CLEARANCE**

SACTCheck applies encoded NCCP protocol criteria to clinician-entered information. This output does not constitute treatment clearance and does not replace complete clinical assessment, the current official NCCP protocol, prescribing information, local policy or professional judgement. The responsible oncology clinician retains responsibility for the final treatment decision.